18.7 Schizophrenia*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "The voices tell me people are poisoning my food." **Objective** • Hears voices • Suspicious; refuses meals • Responds to things others cannot see or hear	**ACTUAL** Impaired verbal communication related to disordered thinking and hallucinations as evidenced by responding to voices and incoherent speech.	• Makes his needs clear and reports that the voices distress him less.	• Use short, clear sentences. • Do not argue with delusions; acknowledge his feelings and present reality gently. • Ask what the voices say, particularly any commands to harm. • Give antipsychotics as prescribed and watch for side effects. • Offer sealed food or let him serve himself if he is suspicious.	• Achieved when needs are communicated.
Subjective • "I prefer to stay alone." **Objective** • Avoids others • Stays in his room	**ACTUAL** Inadequate social connectedness related to withdrawal and suspiciousness as evidenced by avoidance of others.	• Joins one group activity a day.	• Start with one-to-one contact. • Introduce small groups gradually. • Recognize each time he takes part.	• Outcome reached when he participates.
Subjective *No specific complaint* **Objective** • Command hallucinations • Agitated	**RISK** Risk for other-directed violence as evidenced by risk factors: command hallucinations and suspiciousness.	• No one is harmed.	• Watch for early signs of agitation. • Use de-escalation techniques. • Follow the rapid tranquilization policy if ordered.	• Considered met if no violence occurs.

*References for this care plan: [31], [35], [69], [74]. Full entries are listed in the References section of the CPMN book.